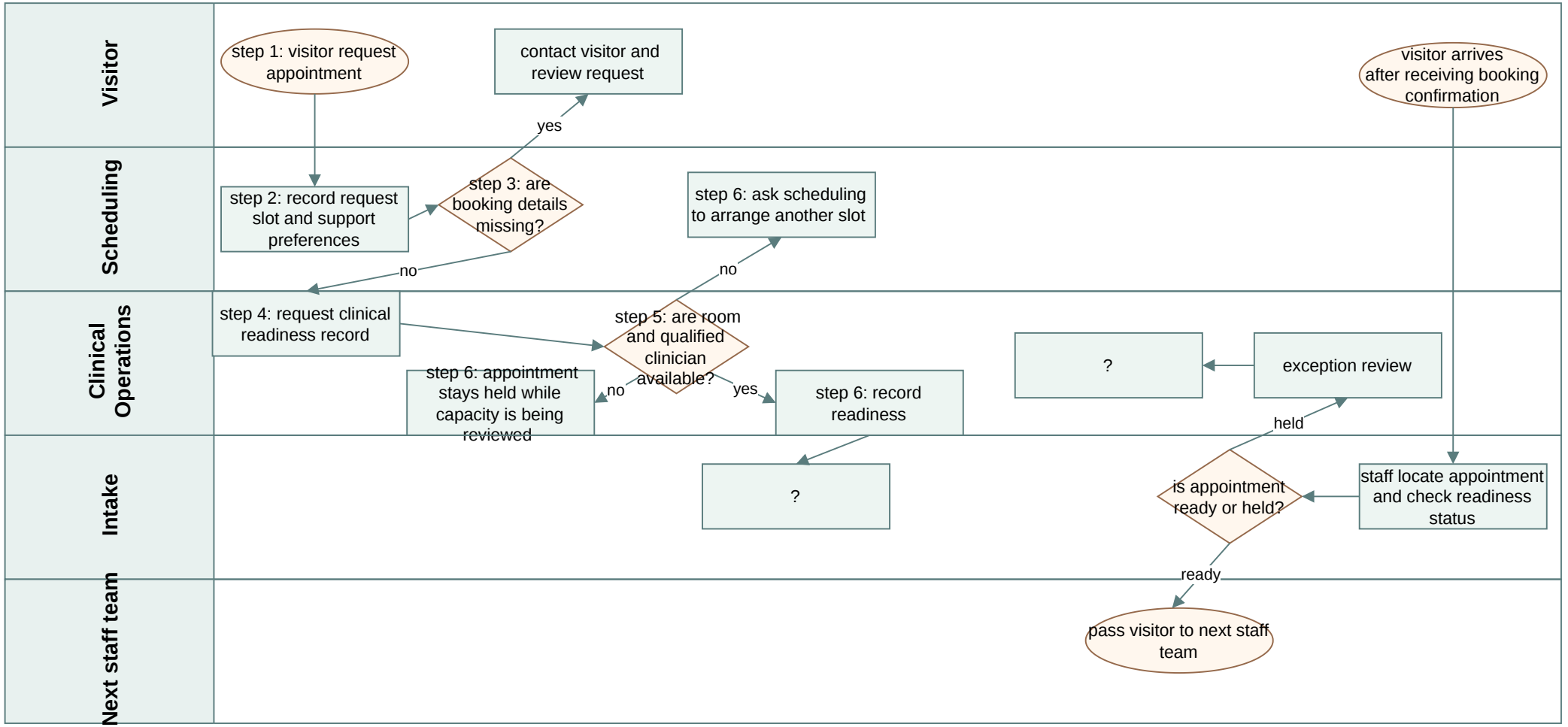
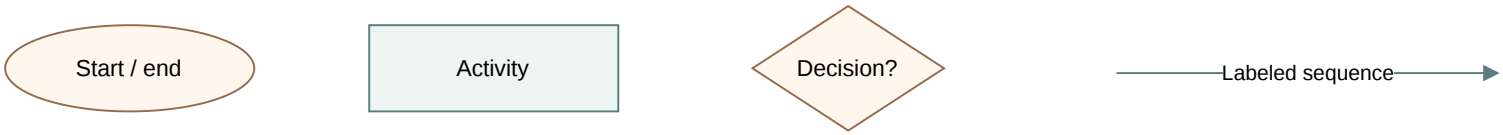
CIT 140 · Week 5 | Current Appointment Process



Symbol key (copy these shapes into your lanes; they are not case steps)



Week 5 | Scope and Findings

1 · Process scope [Your name. Process purpose, start, end, included roles, and what falls outside your diagram.]
The first pathway shows the cycle that happens when a appointment is booked, starting from when the visitor books the appointment with Mateo Silva, the appointment scheduling coordinator, the booking going to scheduling to confirm details , then sent to clinical operations to record readiness if necessary elements of the appointment are available. The second pathway that happens is showing when an appointment is booked and how it either leads to the visitor being able to complete the appointment or having a blockage due to being unable to confirm the appointment is ready. it starts when the visitor arrives and confirm the appoint with the intake coordinator Dana Ruiz, and the part where the decision is missing after the exception review is talked about by Jordan Kim, IT and privacy analyst, and in the W05, S04 source record.

2 · Check the two paths[Trace an ordinary appointment and the incident appointment through your numbered steps. Explain the gap supported by W05-S04.
An ordinary appointment would go through the steps of booking the appointment with scheduling g to confirm details, then it would be moved on to clinical operations to confirm if the room and clinician is available. Then the visitor would be made aware the appointment is booked, they would arrive and intake would confirm readiness and move the visitor to the staff team. The trace of the incident appointment cannot prove appointment details and direct communication from clinical operations and intake because there is no direct line of communication to fix when clinical operations claims appointment is ready or held and when intake sees that an appointment is held.This is discussed in W04-S03 and W04-S04 where both records highlight how each individual authority can approve and validate access for their department but not other fields so there can be confusion when certain departments approve work but others are left unknown as an action is dependanton another department.

3 · Evidence and model review [List source IDs beside the steps they support. Describe any correction you made after checking the model against the records. List remaining uncertainty and the person or record needed to resolve it. Do not invent stakeholder approval.]
The steps 1-4 of the visitor requesting appointment, to scheduling recording and checking if the appointment is missing details, and whether or not would be sent back to visitor or moved onto scheduling is outlined in W05-S01. W05-S02 outlines steps 5-6, it clinical operations does to receive the request, check availability, and if not retain held status and review capacity, OR return to scheduling for when another slot is available. The uncertainty of this path is what happens after readiness is recorded in clinical records and whether that is the end of the path of the visitor booking the appointment. W05-S03 and W05-S04 outlines what happens when the visitor comes after booking, where intake checks if appointment is ready and then contacting clinical operations for review if not. The uncertainty for this path would be clarification on what exactly happens after exemption review, whether the readiness check happens with intake or clinical operations.

4 · Learning reflection (reuse your Discord paragraph)
[In 3–5 sentences: what became clearer; where responsibility or information is unclear and its evidence; how process modeling differs from Week 4 requirements.]
One thing that became clearer was the steps that takes place in booking an appointment from visitor to clinical operations. Another thing that became clearer was where the error in between clinical operations and intake takes place which is confusing parameters and lack of communication on how to ensure a appointment is ready and not be stuck in limbo being held in intake or clinical operations. Process modeling differs from Week 4 because it paints a visual understanding of the process rather than having employee accounts of the situation to go off of, which helps in understanding the issue.

5 · Portfolio evidence
[Editable model filename and Week05/Evidence/Week05_Discord_Post.png or .jpg. If Discord is unavailable, identify the equivalent reflection used here.]
Save the .drawio file; export both pages as one PDF; check readability; commit and push.
Discord was unavailable, equivalent reflection used here